

Lisa King (DOB 24.01.1964)

Date: 04.09.2025 | **Clinic:** Hem/Onc | **MRN:** AML028

Dx: First diagnosed 20.03.2023 with AML, ELN 2022 **Intermediate**, molecular **WT** for FLT3/NPM1/IDH; normal karyotype.

1L regimen: **Azacitidine + Venetoclax** (start **28.03.2023**), completed 12 cycles, now **off therapy** on surveillance.

S: 61F here for routine f/u. Energy “pretty good,” walks 30–40 min daily. No fevers/chills, epistaxis, melena, night sweats, or unintentional wt loss. Notes intermittent constipation early in the month, controlled with PEG. No neuropathy. Vaccines up to date. No new meds.

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- **VS:** BP 124/76, HR 72, afeb, SpO₂ 99% RA, wt stable. ECOG 0.
- **HEENT:** OP clear; sclera anicteric. **Nodes:** none.
- **Chest:** CTAB; **CV:** RRR, no murmurs.
- **Abd:** soft, NT/ND, no HSM.
- **Skin:** no petechiae/ecchymoses.
- **Line:** none (port removed 05.2024).

Labs (04.09.2025):

- **CBC:** WBC 4.8 (ANC 2.3), Hgb 12.7 g/dL, Plt 238×10⁹/L, MCV 92.
- **CMP:** Na 139, K 4.1, Cr 0.88, AST/ALT 21/18, TBili 0.6.
- **LDH:** 175.
- **Coag:** INR 1.0.
- **Molecular/MRD:** Multiparameter flow negative (03.2025). Targeted NGS (peripheral blood) without emergent myeloid driver. qPCR panel (institutional, fusion-agnostic) negative.

Prior course/AE mgmt:

- Cycles 1–2: transient grade 3 neutropenia day 21 → short course **G-CSF** post-cycle; no febrile episodes.
- Cycles 3–12: maintained venetoclax D1–21; antimicrobial prophylaxis during nadirs (levofloxacin/acyclovir); **azole avoided** to preserve ven dosing.
- After 12th cycle (03–04.2024): marrow CR with MRD-negative flow; counts normalized → plan changed to **observation**.

PMH/Concomitant meds:

- Primary hypothyroidism (**levothyroxine 75 µg daily**), hypercholesterolemia (**atorvastatin 20 mg qHS**), GERD (**famotidine PRN**). No anticoagulation.

A: AML in continued remission off therapy; clinically and hematologically stable. No evidence of relapse on serial assessments since completing one year of ven/aza. Good functional status, no significant late effects identified.

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1. **Surveillance only.** CBC q6–8 weeks × year 3, then space to q3 months if stable. Bone marrow only if cytopenias or abnormal smear.
2. **Preventive care.** Continue vaccinations per schedule; annual influenza this month; COVID booster when available. Dental cleanings with caution if counts fall (call us first).
3. **Meds/Interactions.** Avoid strong CYP3A modifiers unless discussed; though off venetoclax, we still counsel to flag any new meds.
4. **When to call:** $T \geq 38.0^{\circ}\text{C}$, new bruising, gum bleeding, persistent fatigue, or drenching night sweats.
5. **Next visit:** 10–12 weeks with CBC; earlier PRN.

Signature: [Name], NP (with MD supervision)